

eCBHFA Volunteer manual

Module: Voluntary Non-Remunerated Blood Donation

Topic 1: Introduction to blood donation

Overview

In this topic, you will learn about the importance of blood in our bodies, and about different types of blood and their effect on blood transfusion and blood donation.

Learning objectives

By the end of this topic you will be better able to:

- explain the purpose of blood in our bodies
- recount the history of blood transfusion and blood donation
- explain why blood donors are needed

What is blood?

Blood is vital to human life. It is the fluid that carries food and oxygen to every cell in your body to keep it fed and oxygenated. It also takes away carbon dioxide and waste from your cells so your body can get rid of them. If you did not have enough blood in your body, your cells and then your whole body would quickly die from not enough water, food and oxygen and too much waste and carbon dioxide.

Blood is a powerful liquid. Every drop of blood is made up of three components, each of which has a critical role. These three components keep the nutrients and oxygen flowing, help you to stop bleeding too much if you get a cut and protect you from bacteria and viruses that may want to hurt you.

The average human adult has eight to ten pints or 3.7 to 4.7 litres of blood in their whole body at one time. A newborn baby has about one cup of blood or 240 millilitres of blood in his or her little body. If more than 15 per cent of the blood is lost (more than half a pint or 0.6 litres in an adult or 36 millilitres in a newborn), whether from bleeding externally or internally, bodies have to work harder to keep the cells fed and oxygenated. People can die from blood loss very quickly as a result of any trauma like an accident, emergency surgery, haemorrhage after childbirth or a diagnosis of leukaemia or dengue (diseases that damage the blood's ability to work properly). Having enough blood is critical in keeping a person alive. It is possible, however, to replace blood loss with the blood of another person. Donated blood can and does save lives.

There is no artificial substitute for blood. Blood donation is the only way to collect this life-saving fluid. However, in many parts of the world, there is not sufficient donated blood to treat everyone who needs it. Worldwide, approximately 112.5 million units of blood are donated each year but this is not enough to meet the many needs for blood¹. For every 1,000 people, 32 donations are given in high-income countries, 15 in upper middle-income countries, 7.6 in lower middle-income countries and only 4.6 in low-income countries. These numbers mean that if you are living in a low- or middle- income country, your community needs more blood donors to address the need for blood.

Reflection

1. Why cannot hospitals just make more blood?
2. What does blood do that is so special?
3. Imagine a container with four litres of water. Does it look like a lot of water? How quickly could you spill a half litre? If you learnt that losing more than one quarter of the water meant you would die, what would you do to ensure you did not lose too much water?
4. What is one new thing you have learnt about blood?

The history of blood transfusion and blood donation

The first recorded blood transfusion took place in the 17th century when the French physician, Jean-Baptiste Denis, infused sheep's blood into a human. It was unsuccessful because he assumed that all blood was the same.

It was not until 1900 that an Austrian scientist, Karl Landsteiner, found that not all blood is exactly the same, even among humans. He found that you are born with a specific blood type that always stays the same throughout your life. The four main blood types are: **A, B, AB and O**.

Scientists learnt that when a person with one blood type receives a blood donation from a person with a different blood type, their body may think it is something harmful and will attack the donated blood, causing even more harm. Some blood types can be given to people with different blood types while others might be able to receive only from certain other types. Knowing these classifications made it safer for people to receive blood from people of the same or certain other blood groups.

Later scientists discovered even more about blood types. They found that Rhesus (RH), a protein, was either present in the blood or it was not. The four blood groups were then classified as Rh-positive or RH-negative, shown as A+ or A-, for example. So there are eight blood groups in total.

The distinction is especially important for pregnant women only if their RH factor is negative and the father's RH factor is positive, as mixing the blood of the foetus and the blood of the mother can cause complications. If you are not sure of your RH factor, complications in pregnancy can be prevented with a series of special injections available from your health provider.

¹ 10 facts on blood transfusion (2017). World Health Organization. Retrieved 9 May 2018 from www.who.int/features/factfiles/blood_transfusion/en/

With all this new information, the following became known:

If your blood type is	You can donate blood to	You can receive blood from
O+	O+, A+, B+, AB+	O+, O-
A+	A+, AB+	A+, A-, O+, O-
B+	B+, AB+	B+, B-, O+, O-
AB+	AB+	All blood types
O-	All blood types	O- only
A-	A-, A+, AB-, AB+	A-, O-
B-	B-, B+, AB-, AB+	B-, O-
AB-	AB-, AB+	AB-, A-, B-, O-

Blood centres and hospitals are fully aware of this guidance, ensuring that when you donate blood, it goes to a person whose blood type will accept your blood type. There is no danger to the donor at all, as your blood is collected and processed, with no-one else's blood coming into contact with you.

You do not even have to know what your blood type is in advance, as it will be tested and typed when you go to donate.

With this knowledge of the blood groups, the RH factor and the components in blood and the function of each, a doctor in Chicago in 1937 started the first blood donation centre. Since that time, the issue of ensuring that blood is safe for transfusion has become more and more of a challenge. Today it is right at the forefront of the practice of blood transfusion therapy.

Reflection

1. Why is it important that medical staff know about blood types?
2. Do you need to know your blood type to donate blood?
3. Do you need to know your blood type to receive blood?
4. What happens if you get the "wrong" blood type?
5. If you are pregnant, what do you need to know about the RH factors of your and your partner's blood?

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Topic 2: Voluntary, non-remunerated blood donation

Overview

In this topic, you learn the definition of voluntary, non-remunerated blood donation and how voluntary, non-remunerated blood donors are chosen according to specific selection criteria designed to ensure the blood taken is safe.

Learning objectives

By the end of this topic, you will be better able to:

- Define voluntary, non-remunerated blood donation
- Explain what makes a person ineligible to donate blood

What is voluntary, non-remunerated blood donation?

Voluntary, non-remunerated blood donation (VNRBD) means that blood is collected from people who donate willingly and who do not expect to be paid for their blood or expect anything in return other than a snack or a small thank-you token and the knowledge that they have done something that is good for their community. Voluntary, non-remunerated blood donors are defined as:

Persons who give blood, plasma or other blood components of their own free will and receive no payment for it, either in the form of cash, or in kind which could be considered a substitute for money. This includes time off work, other than reasonably needed for the donation and travel. Small tokens, refreshments and reimbursements for direct travel costs are compatible with voluntary, non-remunerated blood donation.

The International Federation emphasizes the importance of this definition, which was adopted officially in 1991 by the World Health Organization, the IFRC, the Council of Europe, the International Society of Blood Transfusion, the International Federation of Blood Donor Organizations and a number of other international and national organizations. It is now estimated that up to 60 per cent of blood collected worldwide is donated directly or indirectly through Red Cross and Red Crescent blood donor recruitment programmes, which target voluntary, non-remunerated blood donors. Working with the governments and health authorities of each country, the IFRC has helped raise awareness in National Societies of the vital need for the education, recruitment and retention of voluntary, non-remunerated blood donors with altruistic motives.

It is recognized that the safest blood donors are voluntary, non-remunerated donors from low-risk population groups who donate blood because they want to help others, and who make their donations on a regular basis. One of the main reasons why the blood supply is inadequate in many countries is a lack of voluntary, non-remunerated donors who give blood regularly. In developing countries, less than 40 per cent of the blood supply currently comes from VNRBD.

Talking with VNRBD donors, they report that when they give blood, they feel better knowing that they are helping someone. They feel that they are contributing to their community and that if they need blood one day, it will be available from other donors who give blood in the same good spirit. Donors give blood for a variety of reasons – perhaps because there has been an appeal when there is a blood shortage, perhaps to honour a loved one, perhaps to go along with friends who donate. The reasons are endless, but the main motivation should never be about money since the donor then has an incentive to not be completely truthful about his/her health status and health behaviours.

VNRBD donors are recognized globally as heroes. Every time they donate blood, they are intentionally saving someone's life, not normally someone they know, but who is part of their local or national community.

Reflection

1. Why is it so important that blood donors are not paid?
2. How does voluntary blood donation usually result in healthier donors and safer blood donated?
3. Visit your local blood centre or their website. Find out in your country, what is the approximate percentage of:
 - paid donors?
 - family/replacement or coerced donors? These are donors who donate only to a family member, or who give a donation to replace one used by a family member, or are put under pressure to donate
 - voluntary, non-remunerated donors?
4. Is there a law in place in your country forbidding the sale of blood or the payment of blood donors? If so, how is the law implemented? Is it effective?
5. If not, what are the main reasons why it has not yet been possible to prevent paid blood donation?

Who is eligible to donate blood?

To comply with the safety standard, only suitable donors are allowed to actually donate. Suitable donors meet the donor selection criteria set by your government or blood centre. Selection criteria vary from country to country, but they are likely to include:

- minimum and maximum age for donating
- healthy weight of at least 110 pounds or 50 kilograms
- meeting basic health requirements

While anyone can show up to donate, the process helps to identify who meets the criteria and ensures only the safest donors' blood is taken. Basic health issues include people whose health or personal behaviours may put them at risk of having blood that is not acceptable for donation. Health issues and at-risk behaviours may include:

- intravenous drug use
- anaemia or lack of enough iron in your blood
- current pregnancy
- diagnosis of certain diseases like HIV/AIDS, certain types of cancer, influenza, allergies, syphilis, autoimmune diseases, malaria or hepatitis
- recent surgery
- unprotected sexual activity among multiple partners
- prostitution
- tattooing, piercing and/or blood rituals
- use of non-sterile sharps or knives or blades
- engaging in sexual activity with people who practise risky behaviours
- travel to certain places

In general, if you are fit and well and have not been sick recently, you are strongly encouraged to donate. You will feel so good about helping others in need and you will even save some lives – all from the comfort of the donation centre.

Over the next topics, you will learn more about blood and how it works, about the benefits of VNRBD and about what happens when you donate blood.

Reflection

- Why are people excluded from giving blood if they have been diagnosed with certain diseases or health issues?
 - Why are people excluded if they weigh less than 110 pounds or 50 kilograms?
 - Why is there an age limit for giving blood?
 - If someone practises some of these risky behaviours, will their blood still be given to a patient?
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Topic 3: Components of blood

Overview

In this topic, you learn about the components of blood, what each does for your body and which type of injuries or diseases use different components for healing.

Learning objectives

By the end of this topic, you will be better able to:

- Describe the main components of blood and explain why components are usually separated after donation
- List the main uses for each blood component and the lifespan of each

Whole blood

In modern medical treatments, patients may receive a unit or units of whole blood or just specific components of the blood needed to treat their health condition. Up to three components can be produced from donated blood. By separating the blood into components, several patients can benefit from one unit of donated whole blood¹.

Whole blood is still frequently used for transfusion, particularly in places where blood services have limited facilities and resources, but the processing of units of whole blood into separate components is becoming more common. Most patients need only one component or product, so by separating into components, one unit of whole blood can be used for more than one patient.

If whole blood is transfused into every patient, fewer lives would be saved. One unit of blood broken into components can save three lives, but when it is not separated it can only save one life. Some patients who have survived a very bad accident or are undergoing surgery or a treatment may require many more than one unit. For example, a person who is having a long surgery may lose a lot of blood during the surgery and may need up to 100 units. This is why more donors are needed and why there is a need to use blood wisely whenever possible by separating it into components.

¹ Blood Components | What is in Blood | American Red Cross. (2018). American Red Cross. Retrieved 21 February 2018, from <https://www.redcrossblood.org/learn-about-blood/blood-components>

Study the blood components tool. You can see that every drop of whole blood is made up of three main components:

- Plasma
- Platelets
- Red blood cells

Blood and blood products are also used to prevent health conditions that might cause illness or death. Every minute of the day, blood products are being used for people who are exposed or at risk.

Plasma

Plasma makes up 55 per cent of every drop of blood. Plasma is the fluid in blood which helps the other components to flow. Plasma carries all the other cells as well as other substances like proteins, clotting factors and other necessary elements.

A number of blood products, which have specific uses, can also be derived from plasma.

Platelets

Platelets make up a small part of whole blood – less than one per cent. Platelets help to control bleeding after an injury. Without platelets, the blood would just keep flowing out from a cut or wound, risking a person dying from blood loss. The special job of platelets is to stick to the injured surfaces of the blood vessels, and allow clotting factors in the blood to gather at the injury and clot the blood to stop it from leaving the body.

Red blood cells

Red blood cells make up 45 per cent of whole blood. They carry oxygen from the lungs to the body's cells and carry the used oxygen in the form of carbon dioxide back to the lungs to be exhaled. White blood cells are a small part of blood and defend the body against disease. They make antibodies and fight infections but they are not generally given to patients as part of a blood transfusion except in some rare circumstances.

The table below gives some examples of blood components and their main uses. This is not a comprehensive list but gives a brief indication of the principal components.

	Whole blood	Blood platelets	Red blood	Plasma
Shelf life	21-35 days	5 days	Up to 42 days	Can be frozen up to one year
Key uses	▪ Trauma ▪ Surgery	▪ Cancer treatments ▪ Organ transplants	▪ Trauma ▪ Surgery ▪ Anemia ▪ Any blood loss ▪ Blood disorders, such as sickle cell anemia	▪ Burn patients ▪ Shock ▪ Bleeding disorders

Blood transfusion is regularly used in cases of surgery, trauma, gastrointestinal bleeding and childbirth to replace major blood loss. Cancer patients also often require blood transfusions. Some genetic disorders affect the blood, such as thalassaemia and sickle cell anaemia. Persons with these disorders require regular supplies of safe blood to replace their deficient blood.

The blood used for transfusion must come from healthy people with healthy blood. Blood is not only a living tissue but also a renewable one, with healthy people having the mechanism to make more blood fairly quickly. By donating blood, the gift of life can be given easily.

Reflection

1. Some blood components like platelets only have a shelf life of five days. Why is it so important then to give blood regularly?
 2. Why do countries with fewer resources usually only use whole blood?
 3. If a country only uses whole blood, why does this mean that fewer people's lives can be saved?
 4. Do you know anyone with any of the health conditions listed? Do they require regular blood transfusions?
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Topic 4: Blood donors are heroes!

Overview

In this topic, you will learn about how blood donors and blood recipients both benefit from VNRBD and you will explore some fears and concerns about blood donation in order to know how best to motivate community members to become voluntary blood donors.

Learning objectives

By the end of this topic, you will be better able to:

- Explain fully the importance of VNRBD
- Explain the benefits of being a blood donor
- Explain the benefits of blood donation for recipients
- Discuss some common fears and concerns about blood donation
- Motivate others to donate blood

Voluntary non-remunerated blood donation (VNRBD)

In the second topic of this module, we explored the definition of VNRBD. Here you will learn more about Red Cross and Red Crescent's goal to reach 100 per cent VNRBD for the safest and most reliable blood supply. Today, out of a total of 190 National Societies, some are responsible for major blood programmes at the national level. Others have complete blood services or are involved in blood centre activities. Most are involved in promoting blood donation and blood donor recruitment. Check with your National Society and learn the location of the nearest blood donation centre.

The reasons for promoting 100 per cent voluntary, non-remunerated blood donation are:

- **Protection of blood recipients.** Voluntary blood donors are usually very healthy with their blood free of disease. Because they are not paid and have little reason to withhold honest information about their health status or behaviour that may make them unacceptable as donors, they are open about any potential issues. This makes it easier to screen unsuitable donors out. Repeat donors are those who have donated before and come back regularly to donate blood. They are generally safer than new donors because they are better informed about the importance of low-risk behaviour and the importance of not donating blood whe

they have been sick or practising unhealthy or risky behaviours. Because they donate to help people, they will not donate if there is a chance their blood is not 100 per cent healthy.

- **Protection of the donor.** The blood service has a duty to protect the health and well-being of the donor as well as the recipient. Donors who give blood for payment or reward may hide information that would otherwise cause them to be rejected as a donor temporarily or permanently. This applies equally to those under pressure to donate blood, such as in the common practice in many countries of donating when a family member or a friend needs blood. This is called family/replacement donation and donors who give in this way are not included among voluntary non-remunerated blood donors.
- **Ethically correct.** The International Federation, the World Health Organization and many other organizations believe that it is morally unacceptable for any aspect of health care to be based on the purchase of body parts, including blood. While some recipients may pay a fee for blood received, the fee covers the costs of blood facilities, blood processing, blood centre staff costs and transport of blood to hospital. The fee is not for purchasing the blood or paying a donor.

In 1975, the payment of donors was condemned by the World Health Assembly as "exploitation of the poor". Since then, there has been a drive to eliminate physical incentives like money and gifts since people who sell their blood are typically less healthy than those who donate voluntarily. Their motivations are very different. Blood taken from paid donors is less likely to be healthy than a volunteer's blood.

It is the responsibility of a healthy community to encourage voluntary non-remunerated blood donation so that community members can take care of each other in their greatest time of need and ensure a healthy blood supply for all.

How does voluntary non-remunerated blood donation help the donor?

- Giving blood is one of the greatest acts of humanity. It is a commitment to one's own community by showing solidarity with the local community and across all of humanity.
- In a system based on voluntary blood donation, the health and well-being of the donor is as important as the health and well-being of the recipient.
- Some blood centres thank their voluntary blood donors with special t-shirts, badges, certificates and/or small, non-cash tokens of appreciation. They recognize donors as being special. Only voluntary blood donors receive these items – they cannot be purchased.
- Voluntary blood donors feel good about themselves and feel release from social pressure to donate. People may understand that donating blood is easy yet they often find excuses not to donate. By giving a little time, donors know that they are saving lives.
- Blood from voluntary blood donors often goes to help women in difficult childbirth deliveries. Your blood can save a new mother and a newborn child, contributing to the growth and well-being of your community.
- VNRBD is seen as a virtuous act in many cultures as it shows respect for loved ones. In some countries, voluntary blood donation is seen as a commitment to nation. The unselfish and generous act of giving of yourself would be in agreement with the teachings of most religions.

How does voluntary non-remunerated blood donation help the recipient?

- Taking blood only from voluntary, non-remunerated blood donors means that the blood is more likely to be safe to be transfused, as these donors have no reason to provide inaccurate details about their health status.
- Many people would not be alive today if it were not for the generosity of voluntary non-remunerated blood donors who donate from altruistic motives.
- The blood of paid donors, when tested, has higher rates for infectious disease markers than unpaid donors.
- Sometimes a newly-recognized disease is identified which can be transmitted to others through blood. It has been the case in the past that the levels of positive markers for this new disease were found in many more units of the blood in paid donor blood centres than in the blood from voluntary blood donation centres.
- Even when paid donor blood centres apply all available testing procedures, it is still most likely that their donor population is at greater risk of less healthy blood.

Reflection

1. Why is voluntary blood donation critical to a healthy blood supply?
2. Why is non-remunerated blood donation critical to a healthy blood supply?
3. Why do VNRBD donors generally have safer blood than paid blood donors?

How does voluntary non-remunerated blood donation contribute to a healthy community?

Voluntary non-remunerated donors are more likely to be healthy, follow a healthy lifestyle and contribute to a healthy community. Thus, the needs of a National Society or government blood service are linked to the needs of the wider community. Programmes such as sex education campaigns and support for health promotion activities become part of a long-term strategy to establish a base of safe blood donors.

Programmes for HIV/AIDS prevention and control help build up low-risk population groups, and education in schools about good nutrition, hygiene and healthy lifestyles prepares the way for healthy blood donors of the future. National Societies, non-governmental organizations, such as voluntary blood donor associations, social societies, community organizations, religious groups and even the media can play an important role in promoting a healthy lifestyle. Blood donors themselves are an excellent change agent giving positive messages about healthy lifestyles to the wider community.

Reflection

1. How many units of blood are donated in your country each year?
2. Is the blood supply enough? How much more blood is needed?
3. How can you, as an eCBHFA volunteer help?
4. How many of the blood donors are voluntary?
5. How many blood donations are paid?

Beliefs about blood

Wherever you are in the world, blood has a very important cultural meaning. It is often seen as a life force, and as a symbol of family and kinship ties. It may even be equated with personality and appearance. There may be superstitions about the giving of blood that will have a negative impact on the rate of blood donations. For example:

- My donated blood can be used to bewitch me.
- My donated blood will allow someone to get power over me.
- The blood centre will sell my blood even though I give it for free.
- Giving blood hurts.
- I will catch a disease if I donate blood.

As an eCBHFA volunteer, you will see how important it is to be aware of cultural sensitivities and to provide accurate information to ensure that your VNRBD behaviour change campaigns address any of these kinds of issues believed by your community.

There are numerous factors that can demotivate people who have never donated blood before. Overcoming negative feelings about blood donation is a major step. The challenge for you is to find the key that will work in your culture or environment.

You can use the fact flow game kit (provided) to dig into some of the beliefs about blood in your community and have some fun at the same time. The game provides the correct information to address fears and concerns about blood and donating blood. It can be played by two to four people at the same time, so is an ideal tool to help you work with small groups of community members to dispel wrong beliefs, to provide good information and to motivate them to become voluntary non-remunerated blood donors.

Another good way of motivating community members to become donors is to tell them some real-life stories about blood donors and recipients. Three stories are provided but you might explore other stories from people in your own community who have donated blood voluntarily, who have received life-saving blood or whose loved ones have died because of a lack of blood. Use the stories and the reflection questions to explore beliefs, worries and fears about blood and blood donation so that you can approach the subject in the most culturally sensitive way.

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Topic 5: Let's donate

Overview

In this topic, we learn that there are many reasons why people practice unhealthy behaviours. People are affected by access to services or facilities, social norms and influences where they work, live or play. Behaviour change is the study of how and why people change some habit or action in their life. As volunteers, we have to understand WHY the behaviour is happening and WHAT actions will create change to create behaviour change.

Learning objectives

At the end of this topic, you will be able to define:

- An unhealthy behaviour
- The steps of behaviour change
- The goals of behaviour change
- The main challenges of behaviour change

Tools

- Elephant poster

Topic Content

Volunteers play an important role in helping community members to make healthy changes in their lives. Volunteers help communities to identify problematic behaviours and link the elements of behaviour change together to achieve the behaviour change goals.

This module gives a simple overview of the research behind effective behaviour change. As a volunteer, you will learn about the stages of behaviour change, how behaviour change occurs at different levels, and how change occurs within a theory of change.

Many factors are involved when we try to understand why people practise unhealthy behaviours. These factors may include a person's environment such as adverts they see for unhealthy items or social norms which are patterns of how our friends and family do and think. These factors are linked with how people think and behave.

Behaviours are not always responses that we can control. They may be voluntary or they may be reactions or movements made in response to people in specific situations. Unhealthy behaviours are those that increase our risks of developing illnesses and may lead to poor health or even death. Examples of unhealthy habits include tobacco use, unhealthy diet, inactivity, failure to practice safe sex or failure to wash our hands at critical times. Healthy behaviours are those that prevent illnesses and can improve our health. Examples of healthy behaviours include handwashing, breastfeeding, immunizations, consistent condom use, and use of bednets. The problem in behaviour change is that healthy change is often difficult to implement and maintain.

Behaviour change is the study of how and why people change some habit or action in their life. It is a critical element when considering people's unhealthy and healthy behaviours. Behaviour change includes research from sociology and psychology. This research shows that when we want to change a behaviour, we must consider WHY the behaviour is happening and put in place actions that will create change.

Reflection

1. Why is the science of behaviour change important for volunteers to understand?
2. List an example of an unhealthy behaviour you or a family member practices.
3. How have you tried to address this unhealthy behaviour in the past? Was there a change in behaviour? What did you do next?
4. List an example of a healthy behaviour you practice?
5. How is behaviour change MORE than simply sharing health messages?

So how do we change behaviour? – Elephant poster

Jonathan Haidt, an American researcher who studies why people do the things that they do came up with a great story to explain why we do the behaviours that we do. He talks about how each of us is the whole picture of an elephant, a rider and a path. Our brains are the rider – they are very tiny compared to all the other pieces of this concept, but our rider – or our thoughts - largely make the decisions in our lives. Our Path is the environment where we live, work and play. This is all that surrounds us both physically, emotionally and mentally that may keep us from getting to where we want to go OR help us get there faster. Finally the Elephant is our Motivation or our Drive. It is big and strong and nobody can tell it what to do. While our Elephants might be tamed for awhile like by going on a diet for a few days or stopping smoking for a week or so, the Elephant is big and mighty and can only be stopped or encouraged when properly motivated.

We can use this story to explain how and why we do the behaviours that we do.

How do volunteers develop a behaviour change plan?

The general process for developing a behaviour change intervention includes staff and volunteers working through the general steps of:

1. Sensitising the community to the behaviour change process using the theory of change model.
2. Assessing the problem behaviour – why it is practiced, who practices it, when it is practised, what factors in the environment or society encourage the behaviour. Assess this information at the different levels of the social-ecological model for each community you serve.
3. Identifying an appropriate behaviour goal based on your assessment.
4. Reviewing the causes or barriers at each level that allow the behaviour to continue. Identify interventions that align with each cause or barrier and that can be used at different levels.
5. Discussing the suggested interventions for each SEM level with the community.
6. Identifying appropriate interventions for the context at each level. Interventions should be planned to address the stages of the Theory of Change by first giving knowledge and addressing environmental factors, motivating key people to gain approval and intentions, and ultimately inciting people to action that contributes to the overall goal.
7. Implementing the interventions at each level
8. Monitoring to see if change is happening. Change takes time but it must be monitored to ensure that it is changing, even slowly. Additionally, as people go through the change process, their barriers and causes will change. The behaviour change interventions should adjust to these changes to ensure that change can continue.
9. Recognising that when change is not happening as intended, do further assessment and intervention tweaking.
10. Continuing to implement, monitor, assess, adjust as the change process happens.

Reflection

1. The steps of behaviour change align with the project cycle. Identify each project cycle stage in the steps above.
2. How can you identify causes of behaviour? How can you identify barriers to good behaviour?
3. Why is the behaviour goal done after assessment?
4. Why would a behaviour change plan need to be adjusted during implementation?

Challenges

While assessing and planning for behaviour change, we encounter four major challenges:

1. Identifying why people continue unhealthy behaviours
2. Identifying the exact behaviour goal
3. Choosing the appropriate methods to change behaviour
4. Adequate implementation of the intervention

The most frequent intervention failures in any project including behaviour change processes include:

- Inappropriate choice of methods or interventions
- Failure to address the different levels of SEM with tailored interventions
- Poor implementation of the chosen behaviour change method
- Failure to monitor and tweak interventions as needed throughout the life of the project

The purpose of developing a behaviour change lens for any project is to ensure that each step is done appropriately to ensure behaviour change results. As a volunteer, you are perfectly placed to provide motivation, feedback and monitoring of the programme to ensure success.

Reflection

1. Which of the challenges listed have you seen done in prior CBHFA programmes?
2. How can volunteers track the different pieces of information of a behaviour change-based project?
3. How is monitoring a behaviour change project different from evaluation?

Wrap-Up

Behaviour change is a complex but critical process in any CBHFA activity. While it may seem at first to be difficult, as a community member, volunteers have lots of knowledge that will help to make behaviour change more effective.